*** Facesheet - Auth (Verified) ***

Encompass Health Rehabilitation Hospital of Desert Canyon 7022527342

Encounter Information

FIN: 501361583
Admit Dt/Tm: 02/16/2026 19:35
Disch Dt/Tm:
Attend Physician: Qurashi MD, Munawar A
Admit Physician: Qurashi MD, Munawar A
PCP:
Referring Physician: Shuja MD, Amir
Onset of Symptoms/Illness: (11) 02/05/2026

:

Third Party MRN:
Patient Type: Inpatient
Medical Service: CVA Stroke Program
Subspecialty:
Location: NURS DC
Room/Bed: 208 / P
Admit Reason: STROKE
Accommodation: General Private
VIP Indicator:

Patient Information

Patient Name: LAMBERT, BARBARA J
MRN: 875003035
Home Address: 9420 W WIGWAM AVE UNIT 334
LAS VEGAS, NV 891782478
County: CLARK
Home Phone: (702)355-7597

Admin Sex: Female **Birth Sex:** Female
DOB: 02/16/1958 **Age:** 68 Years
Religion: No Religious Preferenc
Race: Black or African Ameri
Email Address: morrislambert0@gmail.com
Mobile Phone:

Guarantor Information

Guarantor Name: LAMBERT, BARBARA J
Billing Address: 9420 W WIGWAM AVE UNIT 334
LAS VEGAS, NV 891782478

Patient's Reltn: SELF
Home Phone: (702)355-7597
Mobile Phone:

Contact Information

Emergency Contact
Contact Name: LAMBERT, MORRIS
Patient's Reltn: Child/Insured Not
Home Phone: (702)806-0577
Mobile Phone: (702) 806-0577

Emergency Contact
Contact Name: LAMBERT, ANGELA
Patient's Reltn: Other Relationsh
Home Phone: (702) 427-8026
Mobile Phone: (702) 427-8026

Primary Insurance

Insurance Name: MCR Medicare
Financial Class: Medicare
Policy Nbr/Group Nbr: 2N11NT5MX98 /
MBI:

Claim Address: MEDICARE
,
Authorization Number:

Secondary Insurance

Insurance Name: TRIM Tricare For Life
Financial Class: Other Govt- VA/Tricare
Policy Nbr/Group Nbr: 430157586 /
MBI:

Claim Address: TRICARE FOR LIFE
,
Authorization Number:

Tertiary Insurance

Insurance Name:
Financial Class:
Policy Nbr/Group Nbr: /
MBI:

Claim Address:
,
Authorization Number:

Comments:
FROM SO HILLS HOSPITAL/IM: DR SHUJA

Printed by: Benecio Admission Liaison, Suesette on 02/17/2026 11:14 t
Registration last updated by: Benecio Admission Liaison, Suesette on 02/17/2026 11:13

History and Physical

DOCUMENT NAME:	History and Physical
SERVICE DATE/TIME:	2/17/2026 15:06 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Qurashi MD,Munawar A (2/17/2026 15:06 PST)
SIGN INFORMATION:	Qurashi MD,Munawar A (2/18/2026 20:18 PST)

History and Physical Rehab

PATIENT:LAMBERT, BARBARA J.
DOB:02/16/1958
MEDICAL RECORD #:875003035
ACCOUNT NUMBER:501361583

DATE OF ADMISSION:02/16/2026
DATE OF SERVICE:02/17/2026
REFERRING PHYSICIAN:MD Amir Shuja MD
ADMITTING PHYSICIAN:MD Munawar A. Qurashi MD

The patient was admitted to my service yesterday here at the Encompass Health Desert Canyon Inpatient Rehab Hospital.

PRINCIPAL DIAGNOSES:
1. Myopathy in the setting of severe sepsis, septic shock with right perirectal and perianal abscess, with necrotizing fasciitis.
2. Status post debridement, incision and drainage, and wound VAC placement.
3. Erythema myelosis.
4. Acute blood loss anemia.
5. Dyslipidemia.
6. COPD.
7. Protein calorie malnutrition.
8. Hypertension.

HISTORY OF PRESENT ILLNESS: A 67-year-old female, she lives with the family, was ambulatory prior, was admitted to the hospital with rectal pain for the past several months with a bowel movement and recently with the bloody stools. She was admitted to this hospital and workup reveals she has necrotizing fasciitis of right perirectal and perianal area with the abscess size of about 5.8 cm and right medial and gluteal/ischiorectal fossa abscess 4.2 x 2.2 cm. She was followed by the surgeon, taken over to the OR for incision and drainage and debridement for necrotizing wound infection. She was placed on antibiotics, followed by the ID. Wound care was continued. Wound VAC was also placed in.

History and Physical

The patient had pericardial effusion. CT chest was done and no further intervention needed. She had echo, which shows ejection fraction with 55-60%. The patient's surgeon, Dr. Chan continued wound care and extensive rehabilitation program as she is significantly weak and she has been on oxygen.

PAST MEDICAL HISTORY: As mentioned above.

PAST SURGICAL HISTORY: As above.

SOCIAL HISTORY: She used to smoke 0.5 pack per day but currently she smokes 1 pack cigarettes a day. No history of any alcohol use.

FAMILY HISTORY: Noncontributory.

ALLERGIES: No known drug allergies.

CURRENT MEDICATIONS: She is on,
1. Augmentin 875 mg 1 tablet p.o. q.12 hours.
2. Norvasc 5 mg p.o. at bedtime.
3. Lasix 40 mg IV daily.
4. Potassium chloride 20 mEq p.o. daily.
5. Hypochlorite topical apply daily.
6. For pain she is taking Tylenol p.r.n. and oxycodone p.r.n. 10 mg every 4 hours.

REVIEW OF SYSTEMS: She is a pleasant female. She is lying in the bed. She is on oxygen. She seems a little anxious. She talks about the wound on the back. I explained to her that we have a wound doctor who will be following the patient. She denies any headache, any dizziness, chest pain, shortness of breath at rest, but with exertion she gets short of breath. She is on oxygen. She denies any back pain. She denies any numbness, tingling sensation. She denies any bladder symptoms. 10 systems are reviewed.

PHYSICAL EXAMINATION:
GENERAL: Alert, awake, and oriented.
VITAL SIGNS: Current vital signs are blood pressure 124/66, pulse 85, respirations 18, temperature 98.3.
HEENT EXAMINATION: Head is atraumatic. Conjunctivae anicteric.
NECK: Supple.
CHEST: Chest sounds decreased respiratory effort.
HEART: S1 and S2 audible.
ABDOMEN: Soft. Bowel sounds positive.
LOWER EXTREMITIES: There are no signs of clubbing or edema noted.
MUSCULOSKELETAL: Upper extremity strength 4+/5. Lower extremities is about 4/5. Sensation seem grossly intact.

LABS: Today as follows: CBC shows WBC 6.9, hemoglobin 8.7, hematocrit 28.5, and the

History and Physical

platelet count is 241. Comprehensive metabolic panel normals are ALT is 33, AST is 9, total protein 5.8.

DISCUSSION AND PLAN OF CARE: Based on post-admission evaluation, the patient is a good candidate for inpatient rehab hospitalization. The patient had perirectal abscess, being treated for sepsis, septic shock, acute renal failure secondary to acute tubular necrosis, COPD, bloody stools, pericardial effusion with protein-calorie malnutrition with impaired balance gait, transfer, ADLs.

REHAB DIAGNOSIS:
Myopathy related to the above, who is currently being admitted to the inpatient rehab facility here yesterday. In addition the following areas:
1. Current functional status of the patient compared to her pre-admission screening evaluation is about min assist with bed mobility, transfers compared to pre-admission screening evaluation is not much different.
2. Medical necessity for inpatient rehab rather than at subacute level of care, continue the medical management and continue transfer medications, continue the wound care and internist will also be following the patient, continue p.o. antibiotics. All this care better served in acute rehab setting rather than patient at subacute level of care.
3. Patient will undergo comprehensive rehab training that includes services of PT, OT working with her at least 3 hours a day 5 days a week and 24-hour nursing care will be provided to her. Physiatrist will supervise her rehab training program and the caseworker will be helping with discharge planning.

PROGNOSIS: Fair.

LENGTH OF STAY IN REHAB: About 2-3 weeks.

DISPOSITION: Home with the family.

D: 02/17/2026 5:06:14 PM CT
T: 02/17/2026 11:42:06 PM CT
Document ID: 340635216 Confirmation #: 4883174

Electronically Authenticated on 02/18/2026 08:18 PM

Munawar Qurashi, MD

Progress Notes

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 2/23/2026 23:05 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Qurashi MD,Munawar A (2/23/2026 23:07 PST)
SIGN INFORMATION: Qurashi MD,Munawar A (2/23/2026 23:07 PST)

Progress Note - Physician II

Patient: **LAMBERT, BARBARA J** **MRN: 875003035** **FIN: 501361583**
Age: **68 years** Sex: **Female** DOB: **2/16/1958**
Associated Diagnoses: **None**
Author: **Qurashi MD, Munawar A**

Results Review

Feels fine
seen was going on wc to therapy this am

Physical Examination:

CHEST: Chest sounds decreased respiratory effort.
HEART: S1 and S2 audible.
ABDOMEN: Soft. Bowel sounds positive.
LOWER EXTREMITIES: There are no signs of clubbing or edema noted.
MUSCULOSKELETAL: Upper extremity strength 4+/5. Lower extremities is about 4/5.

Vital Signs (last 24 hrs)	Last Charted
Temp Oral	98.9 DegF (FEB 23 19:08)
Heart Rate Peripheral	99 bpm (FEB 23 19:10)
Resp Rate	16 br/min (FEB 23 07:59)
SBP	118 mmHg (FEB 23 19:07)
DBP	**L** 51mmHg (FEB 23 19:07)
SpO2	98 % (FEB 23 19:10)

Medications (18) Active
Scheduled: (4)
atorvastatin 40 mg Tab 40 mg 1 tab, Oral, QHS

Progress Notes

ferrous sulfate 325 mg Tab EC 325 mg 1 tab, Oral, BIDmeals
heparin 5000 units/mL Inj Sol 5,000 units 1 mL, Subcutaneous, q12hr
pantoprazole 20 mg DR Tab 20 mg 1 tab, Oral, Before breakfast
Continuous: (0)
PRN: (14)
acetaminophen 325 mg Tab 650 mg 2 tab, Oral, q4hr
acetaminophen 325 mg Tab 650 mg 2 tab, Oral, q4hr
albuterol-ipratropium 2.5 mg-0.5 mg/3 mL Inh Soln (3mL UD) 3 mL, NEB, q6hr
alum-mag hydroxide-simethicone 2400-2400-240mg/30mL Oral Susp (30mL UD) 30 mL, Oral, q4hr
bisacodyl 10 mg Supp 10 mg 1 supp, PR, As Indicated
cloNIDine 0.1 mg Tab 0.1 mg 1 tab, Oral, q6hr
lactulose 20gm/30mL Oral Syrup (30mL UD) 20 gm 30 mL, Oral, BID
loperamide 2 mg Cap 2 mg 1 cap, Oral, q6hr
midodrine 5 mg Tab 10 mg 2 tab, Oral, TIDmeals
naloxone 0.4 mg/mL Inj Sol 0.4 mg 1 mL, IV Push, As Indicated
ondansetron 4 mg Tab ODT 4 mg 1 tab, SL, q6hr
oxyCODONE 10 mg IR Tab 10 mg 1 tab, Oral, q4hr
oxyCODONE 5 mg IR Tab 5 mg 1 tab, Oral, q4hr
polyethylene glycol 3350 Oral Pwdr-Reconstit (17gm) (1EA UD) 17 gm 1 EA, Oral, As Indicated

Data Review:

Labs: **Labs** (Last four charted values)

WBC	7.8	(FEB 22)	8.5	(FEB 19)	6.9	(FEB 17)		
Hgb	L **9.0**	(FEB 22)	L **8.9**	(FEB 19)	L **8.7**	(FEB 17)		
Hct	L **29.0**	(FEB 22)	L **29.7**	(FEB 19)	L **28.5**	(FEB 17)		
Plt	H **551**	(FEB 22)	342	(FEB 19)	241	(FEB 17)		
Na	L **133**	(FEB 23)	135	(FEB 22)	138	(FEB 19)	140	(FEB 17)
K	4.3	(FEB 23)	H **5.6**	(FEB 22)	4.6	(FEB 19)	4.1	(FEB 17)
CO2	26	(FEB 23)	31	(FEB 22)	32	(FEB 19)	32	(FEB 17)
Cl	L **96**	(FEB 23)	L **97**	(FEB 22)	L **93**	(FEB 19)	98	(FEB 17)
Cr	0.83	(FEB 23)	0.95	(FEB 22)	0.74	(FEB 19)	0.61	(FEB 17)
BUN	H **32**	(FEB 23)	H **31**	(FEB 22)	18	(FEB 19)	13	(FEB 17)

Progress Notes

Glucose		94	(FEB 23)	**H 114**	(FEB 22)	93	(FEB 19)	86	(FEB 17)		
Ca	9.1	(FEB 23)	9.7	(FEB 22)	9.5	(FEB 19)	8.8	(FEB 17)			
Alb	3.7	(FEB 17)									
ALP	48	(FEB 17)									
ALT	L **<3**	(FEB 17)									
AST	L **9**	(FEB 17)									

Functional Status Review:

ADL Status Trend
GG0130 Eating: Setup or clean-up assistance - 05 (02/23/26 18:20:00)
GG0130 Eating: Setup or clean-up assistance - 05 (02/23/26 13:40:00)
GG0130 Oral Hygiene: Setup or clean-up assistance - 05 (02/23/26 11:54:00)
GG0130 Oral Hygiene: Independent - 06 (02/23/26 08:00:00)
GG0130 Upper Body Dressing: Independent - 06 (02/23/26 08:00:00)
GG0130 Upper Body Dressing: Dependent - 01 (02/20/26 13:10:00)
GG0130 Lower Body Dressing: Independent - 06 (02/23/26 08:00:00)
GG0130 Lower Body Dressing: Partial/Moderate assistance - 03 (02/22/26 13:00:00)
GG0130 Toileting Hygiene: Partial/Moderate assistance - 03 (02/23/26 10:34:00)
GG0130 Toileting Hygiene: Partial/Moderate assistance - 03 (02/23/26 10:34:00)
GG0170 Toilet Transfer: Supervision or touching assistance - 04 (02/23/26 10:34:00)
GG0170 Toilet Transfer: Supervision or touching assistance - 04 (02/23/26 10:34:00)
GG0130 Shower, Bathe Self: Independent - 06 (02/23/26 08:00:00)
GG0130 Shower, Bathe Self: Supervision or touching assistance - 04 (02/18/26 13:48:00)
Tub/Shower Transfer- OT: Sup (02/17/26 13:00:00)
Tub/Shower Transfer- OT: Sup (02/16/26 23:59:00)

Mobility Functional Status Trend
GG0170 Lying to Sitting Side of Bed Goal: Independent - 06 (02/19/26 08:00:00)
GG0170 Lying to Sitting Side of Bed Goal: Independent - 06 (02/17/26 09:00:00)
GG0170 Sit to Stand: Independent - 06 (02/23/26 09:00:00)
GG0170 Sit to Stand: Supervision or touching assistance - 04 (02/22/26 13:00:00)
GG0170 Chair,Bed to Chair Transfer: Independent - 06 (02/23/26 09:00:00)
GG0170 Chair,Bed to Chair Transfer: Supervision or touching assistance - 04 (02/22/26 13:00:00)
GG0170 Walk 10 Feet: Independent - 06 (02/20/26 08:00:00)
GG0170 Walk 10 Feet: Supervision or touching assistance - 04 (02/19/26 08:00:00)

Progress Notes

GG0170 Walk 50 Feet with Two Turns: Supervision or touching assistance - 04 (02/20/26 08:00:00)
GG0170 Walk 50 Feet with Two Turns: Supervision or touching assistance - 04 (02/19/26 08:00:00)
GG0170 Walk 150 Feet: Not attempted due to medical condition or safety concerns - 88 (02/17/26 09:00:00)

Speech/Language & Cognition Functional Status Trend
Comprehension Mode Functional Status: Auditory, Visual (02/16/26 20:30:00)
Ability to Hear: Adequate - no difficulty in normal conversation, social interaction, listening to TV (02/17/26 13:00:00)
Visual Acuity: Adequate - sees fine detail, such as regular print in newspapers/books (02/17/26 13:00:00)
Health Literacy SILS: Rarely (02/17/26 11:12:00)
BB0700 Expression of Ideas and Wants: Expresses complex messages without difficulty and with speech that is clear and easy to understand - 4 (02/23/26 09:00:00)
BB0700 Expression of Ideas and Wants: Expresses complex messages without difficulty and with speech that is clear and easy to understand - 4 (02/22/26 21:00:00)
BB0800 Understanding Verbal Content: Understands - Clear comprehension without cues or repetitions - 4 (02/23/26 09:00:00)
BB0800 Understanding Verbal Content: Understands - Clear comprehension without cues or repetitions - 4 (02/22/26 21:00:00)
Expression Mode Functional Status: Vocal (02/16/26 20:30:00)
Attention Functional Status - OT: Independent (02/20/26 14:00:00)
Attention Functional Status - OT: Independent (02/19/26 09:00:00)
Memory Functional Status - OT IP: Independent (02/20/26 14:00:00)
Memory Functional Status - OT IP: Independent (02/19/26 09:00:00)
Safety Awareness/Insight - OT: Independent (02/20/26 14:00:00)
Safety Awareness/Insight - OT: Independent (02/19/26 09:00:00)
Simple Problem Solving Func Status OT: Independent (02/20/26 14:00:00)
Simple Problem Solving Func Status OT: Independent (02/19/26 09:00:00)
Complex Problem Solving Func Status OT: Independent (02/20/26 14:00:00)
Complex Problem Solving Func Status OT: Independent (02/19/26 09:00:00)

Discharge Functional Goals
ADL Functional Goals
GG0130 Eating Goal: Independent - 06 (02/23/26)
GG0130 Lower Body Dressing Goal: Independent - 06 (02/23/26)
GG0130 Oral Hygiene Goal: Independent - 06, IND on eval (02/23/26)
GG0130 Put On,Taking Off Footwear Goal: Independent - 06 (02/23/26)
GG0130 Shower, Bathe Self Goal: Independent - 06 (02/23/26)
GG0130 Toileting Hygiene Goal: Independent - 06 (02/23/26)
GG0130 Upper Body Dressing Goal: Independent - 06 (02/23/26)
GG0170 Toilet Transfer Goal: Independent - 06 (02/23/26)
LTG Comb/brush hair Goal: Ind (02/23/26)
LTG Tub/Shower transfer Goal: Ind (02/23/26)
LTG Wash/Dry face Goal: Ind (02/23/26)
LTG Wash/Dry hands Goal: Ind (02/23/26)
Mobility Functional Goals
GG0170 1 Step (Curb) Goal: Independent - 06 (02/19/26)

Progress Notes

GG0170 4 Steps Goal: Independent - 06 (02/19/26)
GG0170 Car Transfer Goal: Independent - 06 (02/23/26)
GG0170 Chair,Bed to Chair Transfer Goal: Independent - 06 (02/19/26)
GG0170 Lying to Sitting Side of Bed Goal: Independent - 06 (02/19/26)
GG0170 Picking Up Object Goal: Independent - 06 (02/19/26)
GG0170 Roll Left and Right Goal: Independent - 06 (02/19/26)
GG0170 Sit to Lying Goal: Independent - 06 (02/19/26)
GG0170 Sit to Stand Goal: Independent - 06 (02/19/26)
GG0170 Walk 10 Feet Goal: Independent - 06 (02/19/26)
GG0170 Walk 150 Feet Goal: Independent - 06 (02/19/26)
GG0170 Walk 50 Feet with Two Turns Goal: Independent - 06 (02/19/26)
GG0170 Walking 10 Ft Uneven Surface Goal: Independent - 06 (02/19/26)
Speech/Language & Cognition Goals
LTG Attention Cognitive OT: Independent (02/23/26)
LTG Memory Cognitive OT: Independent (02/23/26)
LTG Safety Aware/Insight Cognitive OT: Independent (02/23/26)
LTG Simple Prob Solving Cognitive OT: Independent (02/23/26)
LTG Complex Prob Solving Cognitive OT: Independent (02/23/26)
Impression and Plan/Medical Assessment:

Interpretation of Functional Data:

According to my review of the medical conditions and functional status, the patient is progressing toward DC goals as follows:

progressed from [] to [] toward a goal of []

Continue the rehabilitation plan.

Anticipated Discharge Plan
Anticipated Discharge Plan: Home with Home Health (02/19/26 16:43:00)
Expected Discharge Live With: Alone (02/17/26 11:12:00)
Anticipated Discharge Date: 02/25/26 00:00:00 (02/19/26 16:43:00)

Impression and Plan

1. Myopathy in the setting of severe sepsis, septic shock with right perirectal and perianal abscess, with necrotizing fasciitis.
2. Status post debridement, incision and drainage, and wound VAC placement.
3. Erythema myelosis.
4. Acute blood loss anemia.
5. Dyslipidemia.
6. COPD.
7. Protein calorie malnutrition.
8. Hypertension.

Progress Notes

Comprehensive rehab training program
meds/ labs reviewed
Is max a toileting
2/19/25 continue wound care
on po abx
Gait 50 ft sup level
2/20/25 Is sup with transfers
sup with gait
2/21/26 Keep DVT prophylaxis
continue wound care
Gait 50 ft sup level
2/22/26 Is mod a LE dressing
sup with transfers
continue wound care
2/23/26 DVT prophylaxis
sup gait 50 ft fww
will discuss plan of care in team meeting tomorrow

Electronically Authenticated on 02/23/2026 11:07 PM

Munawar Qurashi, MD

Progress Notes

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 2/23/2026 12:55 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Javed MD,Lubna (2/23/2026 13:01 PST)
SIGN INFORMATION: Javed MD,Lubna (2/23/2026 13:01 PST)

lubna woundcare

Patient: **LAMBERT, BARBARA J** **MRN: 875003035** **FIN: 501361583**
Age: **68 years** Sex: **Female** DOB: **2/16/1958**
Associated Diagnoses: **None**
Author: **Javed MD, Lubna**

Subjective

wound care follow up
seen for :
right hip surgical wound POA - using vashe soaked gauze and doing well
right ischial wound surgical POA - vashe packing
HPI:
67-year-old female, was admitted to the hospital with rectal pain for the past several months with a bowel movement and recently with the bloody stools. She was admitted to this hospital and workup reveals she has necrotizing fasciitis of right perirectal and perianal area with the abscess size of about 5.8 cm and right medial and gluteal/ischiorectal fossa abscess 4.2 x 2.2 cm. She was followed by the surgeon, taken over to the OR for incision and drainage and debridement for necrotizing wound infection. She was placed on antibiotics, followed by the ID. Wound care was continued. Wound VAC was also placed in.The patient had pericardial effusion. CT chest was done and no further intervention needed. She had echo, which shows ejection fraction with 55-60%. pt send here for rehab and we are seeing for woundcare
PMH:
as mentioned
social hx :
non smoker
family hx :
reveiwed
ROS:
14 point ros done neg except mentioned

Health Status

Allergies:

Allergies (1) Active	Reaction
No Known Allergies	None Documented

Problem list:
All Problems

Progress Notes

Ability to perform ADL - activity of daily living / 3302763012 / Confirmed
Activity tolerance / 2156821014 / Confirmed

Objective

VS/Measurements

Vital Signs (last 24 hrs)	Last Charted
Temp Oral	98.2 DegF (FEB 23 07:59)
Heart Rate Peripheral	98 bpm (FEB 23 12:55)
Resp Rate	16 br/min (FEB 23 07:59)
SBP	119 mmHg (FEB 23 12:55)
DBP	61 mmHg (FEB 23 12:55)
SpO2	95 % (FEB 23 08:00)

General: No acute distress.
Eye: Pupils are equal, round and reactive to light, Extraocular movements are intact.
HENT: Normocephalic.
Neck: Supple.
Respiratory: Lungs are clear to auscultation.
Cardiovascular: Normal rate.
Breast: No mass.
Integumentary: 1. right hip wound def better and has 100% granulation and measures 6x3x3 cm and has mild drianage
2. right ischial wound measures 3x0.5x8 cm and had drainage and looks like poop may be getting to the wound due to t6he proximity of the wound .
Mental status/ Cognition: Alert, Oriented x 3.
Cranial Nerves: Cranial nerves II - XII grossly intact.

Results Review
Results review

Labs (Last four charted values)

WBC	7.8	(FEB 22)	8.5	(FEB 19)	6.9	(FEB 17)
Hgb	L **9.0**	(FEB 22)	L **8.9**	(FEB 19)	L **8.7**	(FEB 17)
Hct	L **29.0**	(FEB 22)	L **29.7**	(FEB 19)	L **28.5**	(FEB 17)
Plt	H **551**	(FEB 22)	342	(FEB 19)	241	(FEB 17)

Progress Notes

Na	L **133**	(FEB 23)	135	(FEB 22)	138	(FEB 19)	140	(FEB 17)
K	4.3	(FEB 23)	H **5.6**	(FEB 22)	4.6	(FEB 19)	4.1	(FEB 17)
CO2	26	(FEB 23)	31	(FEB 22)	32	(FEB 19)	32	(FEB 17)
Cl	L **96**	(FEB 23)	L **97**	(FEB 22)	L **93**	(FEB 19)	98	(FEB 17)
Cr	0.83	(FEB 23)	0.95	(FEB 22)	0.74	(FEB 19)	0.61	(FEB 17)
BUN	H **32**	(FEB 23)	H **31**	(FEB 22)	18	(FEB 19)	13	(FEB 17)
Glucose	94	(FEB 23)	H **114**	(FEB 22)	93	(FEB 19)	86	(FEB 17)
Ca	9.1	(FEB 23)	9.7	(FEB 22)	9.5	(FEB 19)	8.8	(FEB 17)
Alb	3.7	(FEB 17)						
ALP	48	(FEB 17)						
ALT	L **<3**	(FEB 17)						
AST	L **9**	(FEB 17)						

Impression and Plan

pt with recent prerirectal abscess, treated for sepsis, septic shock, acute renal failure secondary to acute tubular necrosis, COPD, bloody stools, pericardial effusion with protein-calorie malnutrition

wound care dx and plan :
1. right hip and right ischial wound surgical from nec fasc POA

cont with current dressing
pt to follow up with clinic upon dc at spring valley wound carecenter
also keep the ischial wound clean as too close to the area and poop can contaminate the wound
rounds made with the wound care staff

Progress Notes

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 2/23/2026 11:13 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Shuja MD,Reem (2/23/2026 11:14 PST)
SIGN INFORMATION: Shuja MD,Reem (2/23/2026 11:14 PST)

Internal Medicine

Patient: **LAMBERT, BARBARA J** **MRN: 875003035** **FIN: 501361583**
Age: **68 years** Sex: **Female** DOB: **2/16/1958**
Associated Diagnoses: **None**
Author: **Shuja MD, Reem**

Chief Complaint
sepsis with shock, right perirectal perianal abcess , right medial gluteal /ischiorectal fossa abcess, necrotizing fascitis

History of Present Illness
pt worked with therapy this am
has wound drainage. dressing changed, to be seen by wound team later today
potassium now 4.6
denies chest pain sob palptiaons
had 1 episode diarrhea, c diff was negative

Review of Systems
Constitutional: Negative except as documented in history of present illness.
Eye: eomi perrla moist mucos membranes.
Ear/Nose/Mouth/Throat: tm intact.
Respiratory: clear.
Cardiovascular: s1 s2 .
Gastrointestinal: sot nontender +Bs no palp organomeglay.
Musculoskeletal: able to move all extremiteis.
Integumentary: perirectal right wound with drainage
peri ischial wound.
Neurologic: aao times 3 cranial nerves intact no sensory motor deficit .

Health Status
Allergies:
Allergic Reactions (Selected)
No Known Allergies,

Allergies (1) Active	Reaction
No Known Allergies	None Documented

Medications
Current medications: (Selected)
Inpatient Orders

Progress Notes

Ordered

Admit to Inpatient Hospital:
Aerosol Treatment:
Bowel Care Protocol:
Case Management Discharge Plan:
Case Management Initial Assessment:
Chest XR 2 Views:
Complete Data Collection Information Summary:
Consult to Physician:
Consult to Physician:
Consult to Respiratory Therapy:
Diet:
Dietary Supplements:
Dietary Supplements:
Discharge Nursing Depression Screening:
Education Cardiovascular:
External Urinary Catheter:
Fall Precautions:
Generate the Team Conference Patient Update Document:
Heel Offloading:
Incentive Spirometry:
Initiate Initial Plan of Care:
MiraLax: 17 gm = 1 EA, Oral, As Indicated, PRN: Constipation
Ongoing Assessment Adult Rehabilitation:
Oxygen:
Patient/Plan of Care Review:
Protonix: 20 mg = 1 tab, Oral, Before breakfast
Quality of Life Follow Up:
Reposition as needed:
Resuscitation Status:
SCD Knee high:
Team Conference Tuesday.:
Team Conference.:
VTE Advisor:
Vital Signs:
Weight:
Wheelchair Cushion OT:
Wheelchair Cushion PT:
Wound Care:
acetaminophen: 650 mg = 2 tab, Oral, q4hr, PRN: PAIN (Scale 1-10)
acetaminophen: 650 mg = 2 tab, Oral, q4hr, PRN: Temp greater than or equal to 100.5
albuterol-ipratropium 2.5 mg-0.5 mg/3 mL inhalation solution: 3 mL, NEB, q6hr, PRN: Shortness of breath or wheezing
aluminum hydroxide/magnesium hydroxide/simethicone 400 mg-400 mg-40 mg/5 mL oral suspension: 30 mL, Oral, q4hr, PRN: Indigestion
atorvastatin: 40 mg = 1 tab, Oral, QHS
bisacodyl: 10 mg = 1 supp, PR, As Indicated, PRN: Constipation
cloNIDine 0.1 mg oral tablet: 0.1 mg = 1 tab, Oral, q6hr, PRN: Elevated blood pressure
ferrous sulfate: 325 mg = 1 tab, Oral, BIDmeals
heparin 5000 units/mL injectable solution: 5,000 units = 1 mL, Subcutaneous, q12hr

Progress Notes

lactulose: 20 gm = 30 mL, Oral, BID, PRN: Constipation
loperamide: 2 mg = 1 cap, Oral, q6hr, PRN: Diarrhea
low air loss mattress:
midodrine: 10 mg = 2 tab, Oral, TIDmeals, PRN: Other (see comment)
naloxone: 0.4 mg = 1 mL, IV Push, As Indicated, PRN: Other (see comment)
ondansetron: 4 mg = 1 tab, SL, q6hr, PRN: Nausea or vomiting
oxyCODONE 10 mg oral tablet: 10 mg = 1 tab, Oral, q4hr, PRN: PAIN (Scale 7-10)
oxyCODONE 5 mg oral tablet: 5 mg = 1 tab, Oral, q4hr, PRN: PAIN (Scale 1-6)

Suspended

Senna S: 1 tab, Oral, BID
bisacodyl: 5 mg = 1 tab, Oral, Daily

Documented Medications

Documented

albuterol CFC free 90 mcg/inh inhalation aerosol: 1 puff, INH, q6hr RT, PRN: Shortness of breath, 0 Refill(s)
albuterol-ipratropium 2.5 mg-0.5 mg/3 mL inhalation solution: 3 mL, NEB, BID RT, 60 EA, 0 Refill(s)
amoxicillin-clavulanate 875 mg-125 mg oral tablet: 1 tab, Oral, q12hr, for 7 day, 14 tab, 0 Refill(s)
atorvastatin 40 mg oral tablet: 40 mg = 1 tab, Oral, QHS, 30 tab, 0 Refill(s)
bisacodyl 5 mg oral tablet: 5 mg = 1 tab, Oral, Daily, 0 Refill(s)
furosemide 10 mg/mL injectable solution: 40 mg, IV, Daily, 0 Refill(s)
midodrine 10 mg oral tablet: 10 mg = 1 tab, Oral, TIDmeals, PRN: Other (see comment), 270 tab, 0 Refill(s)
oxyCODONE 10 mg oral tablet: 10 mg = 1 tab, Oral, q4hr, PRN: PAIN (Scale 7-10), 0 Refill(s)
oxyCODONE 5 mg oral tablet: 5 mg = 1 tab, Oral, q4hr, PRN: PAIN (Scale 4-6), 0 Refill(s)
potassium chloride 20 mEq oral tablet, extended release: 20 mEq = 1 tab, Oral, Daily, 30 tab, 0 Refill(s)
sodium hypochlorite 0.25% topical solution: 1 app, Topical, Daily, 0 Refill(s).

Medications (18) Active

Scheduled: (4)

atorvastatin 40 mg Tab 40 mg 1 tab, Oral, QHS, Start 02/17/26 21:00:00 PST
ferrous sulfate 325 mg Tab EC 325 mg 1 tab, Oral, BIDmeals, Start 02/19/26 17:00:00 PST
heparin 5000 units/mL Inj Sol 5,000 units 1 mL, Subcutaneous, q12hr, Start 02/17/26 21:00:00 PST
pantoprazole 20 mg DR Tab 20 mg 1 tab, Oral, Before breakfast, Start 02/19/26 6:00:00 PST

Continuous: (0)

PRN: (14)

acetaminophen 325 mg Tab 650 mg 2 tab, Oral, q4hr, Start 02/16/26 20:07:00 PST
acetaminophen 325 mg Tab 650 mg 2 tab, Oral, q4hr, Start 02/16/26 20:07:00 PST
albuterol-ipratropium 2.5 mg-0.5 mg/3 mL Inh Soln (3mL UD) 3 mL, NEB, q6hr, Start 02/17/26 11:38:00 PST
alum-mag hydroxide-simethicone 2400-2400-240mg/30mL Oral Susp (30mL UD) 30 mL, Oral, q4hr, Start 02/16/26 20:07:00 PST
bisacodyl 10 mg Supp 10 mg 1 supp, PR, As Indicated, Start 02/16/26 20:07:00 PST

Progress Notes

cloNIDine 0.1 mg Tab 0.1 mg 1 tab, Oral, q6hr, Start 02/16/26 20:07:00 PST, Stop 03/18/26 20:06:00 PDT
lactulose 20gm/30mL Oral Syrup (30mL UD) 20 gm 30 mL, Oral, BID, Start 02/16/26 20:07:00 PST
loperamide 2 mg Cap 2 mg 1 cap, Oral, q6hr, Start 02/23/26 10:57:00 PST, Stop 03/25/26 10:56:00 PDT
midodrine 5 mg Tab 10 mg 2 tab, Oral, TIDmeals, Start 02/16/26 21:49:00 PST
naloxone 0.4 mg/mL Inj Sol 0.4 mg 1 mL, IV Push, As Indicated, Start 02/16/26 20:07:00 PST
ondansetron 4 mg Tab ODT 4 mg 1 tab, SL, q6hr, Start 02/16/26 20:07:00 PST, Stop 03/18/26 20:06:00 PDT
oxyCODONE 10 mg IR Tab 10 mg 1 tab, Oral, q4hr, Start 02/16/26 21:49:00 PST
oxyCODONE 5 mg IR Tab 5 mg 1 tab, Oral, q4hr, Start 02/16/26 21:49:00 PST
polyethylene glycol 3350 Oral Pwdr-Reconstit {17gm} (1EA UD) 17 gm 1 EA, Oral, As Indicated, Start 02/16/26 20:07:00 PST

Problem list:
All Problems
Ability to perform ADL - activity of daily living / 3302763012 / Confirmed
Activity tolerance / 2156821014 / Confirmed,
Active Problems (2)
Ability to perform ADL - activity of daily living
Activity tolerance

Histories
Past Medical History:
No active or resolved past medical history items have been selected or recorded.
Family History:
No family history items have been selected or recorded.
Procedure history:
No active procedure history items have been selected or recorded.
Social History

Social & Psychosocial Habits

No Data Available

Physical Examination
VS/Measurements
Vital Signs

2/23/2026 08:00 PST	Peripheral Pulse Rate	94 bpm NORM
2/23/2026 08:00 PST	Systolic Blood Pressure	100 mmHg NORM
	Diastolic Blood Pressure	**46 mmHg <LLOW**
	Mean Arterial Pressure, Cuff	64 mmHg
2/23/2026 07:59 PST	Temperature Oral F	98.2 DegF NORM
	Temperature Oral	36.8 DegC
2/23/2026 07:59 PST	Respiratory Rate	16 br/min NORM
2/23/2026 04:15 PST	Peripheral Pulse Rate	81 bpm NORM
2/23/2026 04:15 PST	Temperature Oral F	98.2 DegF NORM
	Systolic Blood Pressure	135 mmHg NORM

Progress Notes

	Diastolic Blood Pressure	71 mmHg NORM
	Mean Arterial Pressure, Cuff	92 mmHg
	Temperature Oral	36.8 DegC
2/22/2026 19:20 PST	Peripheral Pulse Rate	96 bpm NORM
2/22/2026 19:19 PST	Temperature Oral F	99 DegF NORM
	Temperature Oral	37.0 DegC
2/22/2026 19:19 PST	Systolic Blood Pressure	108 mmHg NORM
	Diastolic Blood Pressure	69 mmHg NORM
	Mean Arterial Pressure, Cuff	82 mmHg
2/22/2026 07:34 PST	Peripheral Pulse Rate	91 bpm NORM
2/22/2026 07:34 PST	Systolic Blood Pressure	130 mmHg NORM
	Diastolic Blood Pressure	65 mmHg NORM
	Mean Arterial Pressure, Cuff	87 mmHg
2/22/2026 07:34 PST	Temperature Oral F	98.5 DegF NORM
	Temperature Oral	36.9 DegC
2/22/2026 04:45 PST	Peripheral Pulse Rate	88 bpm NORM
2/22/2026 04:45 PST	Temperature Oral F	98.4 DegF NORM
	Temperature Oral	36.9 DegC
2/22/2026 04:45 PST	Systolic Blood Pressure	124 mmHg NORM
	Diastolic Blood Pressure	65 mmHg NORM
	Mean Arterial Pressure, Cuff	85 mmHg

,

Vital Signs (last 24 hrs)	**Last Charted**
Temp Oral	98.2 DegF (FEB 23 07:59)
Heart Rate Peripheral	94 bpm (FEB 23 08:00)
Resp Rate	16 br/min (FEB 23 07:59)
SBP	100 mmHg (FEB 23 08:00)
DBP	**C** 46mmHg (FEB 23 08:00)
SpO2	95 % (FEB 23 08:00)

, Measurements from flowsheet : Measurements

2/22/2026 01:53 PST	Weight Dosing lbs	90.2 lb

Review / Management

Results review:

Labs (Last four charted values)

WBC	7.8	(FEB 22)	8.5	(FEB 19)	6.9	(FEB 17)
Hgb	L **9.0**	(FEB 22)	L **8.9**	(FEB 19)	L **8.7**	(FEB 17)
Hct	L **29.0**	(FEB 22)	L **29.7**	(FEB 19)	L **28.5**	(FEB 17)

Progress Notes

Plt	H **551**	(FEB 22)	342	(FEB 19)	241	(FEB 17)		
Na	L **133**	(FEB 23)	135	(FEB 22)	138	(FEB 19)	140	(FEB 17)
K	4.3	(FEB 23)	H **5.6**	(FEB 22)	4.6	(FEB 19)	4.1	(FEB 17)
CO2	26	(FEB 23)	31	(FEB 22)	32	(FEB 19)	32	(FEB 17)
Cl	L **96**	(FEB 23)	L **97**	(FEB 22)	L **93**	(FEB 19)	98	(FEB 17)
Cr	0.83	(FEB 23)	0.95	(FEB 22)	0.74	(FEB 19)	0.61	(FEB 17)
BUN	H **32**	(FEB 23)	H **31**	(FEB 22)	18	(FEB 19)	13	(FEB 17)
Glucose	94	(FEB 23)	H **114**	(FEB 22)	93	(FEB 19)	86	(FEB 17)
Ca	9.1	(FEB 23)	9.7	(FEB 22)	9.5	(FEB 19)	8.8	(FEB 17)
Alb	3.7	(FEB 17)						
ALP	48	(FEB 17)						
ALT	L **<3**	(FEB 17)						
AST	L **9**	(FEB 17)						

, Lab results

2/23/2026 04:27 PST	Estimated Creatinine Clearance	34.85 mL/min
	Creatinine Level	**0.83 mg/dL NA**
	Sodium - QST	**133 mmol/L**
	Potassium - QST	**4.3 mmol/L**
	Chloride - QST	**96 mmol/L**
	Carbon Dioxide - QST	**26 mmol/L**
	Urea Nitrogen (BUN) - QST	**32 mg/dL**
	BUN/Creatinine Ratio - QST	**39 (calculated)**
	Creatinine - QST	**0.83 mg/dL**
	Glucose - QST	**94 mg/dL**
	Calcium - QST	**9.1 mg/dL**
	eGFR CKD-EPI - QST	**77 mL/min/1.73 m2**
2/22/2026 05:20 PST	**White Blood Cell Count - QST**	**7.8 x10(3)/mcL**
	Red Blood Cell Count - QST	**3.60 x10(6)/mcL**
	Hemoglobin - QST	**9.0 g/dL**

Progress Notes

Hematocrit - QST	**29.0 %**
MCV - QST	**80.6 fL**
MCH - QST	**25.0 pg**
MCHC - QST	**31.0 g/dL**
RDW - QST	**19.6 %**
Platelet Count - QST	**551 x10(3)/mcL**
MPV - QST	**10.3 fL**
Absolute Neutrophils - QST	**4267 cells/mcL**
Absolute Lymphocytes - QST	**1763 cells/mcL**
Absolute Monocytes - QST	**1427 cells/mcL**
Absolute Eosinophils - QST	**250 cells/mcL**
Absolute Basophils - QST	**94 cells/mcL**
Neutrophils - QST	**54.7 %**
Monocytes - QST	**18.3 %**
Eosinophils - QST	**3.2 %**
Basophils - QST	**1.2 %**
Lymphocytes - QST	**22.6 %**
Estimated Creatinine Clearance	34.85 mL/min
Creatinine Level	**0.95 mg/dL NA**
Sodium - QST	**135 mmol/L**
Potassium - QST	**5.6 mmol/L**
Chloride - QST	**97 mmol/L**
Carbon Dioxide - QST	**31 mmol/L**
Urea Nitrogen (BUN) - QST	**31 mg/dL**
BUN/Creatinine Ratio - QST	**33 (calculated)**
Creatinine - QST	**0.95 mg/dL**
Glucose - QST	**114 mg/dL**
Calcium - QST	**9.7 mg/dL**
eGFR CKD-EPI - QST	**65 mL/min/1.73 m2**

Impression and Plan

68 year old female

right perirectal /perianal abscess
right medial gluteal /ischiorectal fossa abcess
sever sepsis with shock on arrival to hosptial
acute renal failure
necrotizing fascitis s/p surgery by dr chan
hypokalemia
hyperglyciemia
copd
dyslipidemia
hypotension
protein caloirie malnutrition
pleural effusion

lipitor 40
albuterol
augmentin 875 bid for 3 more days

Progress Notes

midorine 10 tid
oxycodone
dulcolax chabged to prn
colace changed to prn
sq heparin 500 q 12
add prtonix
lasix 40 iv , change to lasix 20 po
kcl 20

pt ot
labs as needed
dvt and gi ppx
fall and aspiration precauions
surgical f/u
wound care
id eval
full code

feb 18 2026 change iv lasix 40 to lasix 20 po for pleural effusion , will recheck cxray , and see if we need to continue lasix, check k level am , id eval
feb 19 2026 pt works with therapy , cxray pleural effusion min , will dc lasix and kcl , add ferrous sulphate , pt ot
feb 20 2026 cont present tm pt ot wound care
feb 21 2026 multiple episodes of loose stool / diarrhea , check stool for c diff , wound care pt ot check am labs
feb 22 2026 lokelma 10 times 1 recheck bmp am , pt ot awaiting c diff result
feb 23 2026 potassium normal , imodium for diarrhea, c diff negative, wound care f/u ptot

Electronically Authenticated on 02/23/2026 11:14 AM

Reem Shuja, MD

DOC - Occupational Therapy

DOCUMENT NAME: IP OT Daily Documentation - Text
SERVICE DATE/TIME: 2/23/2026 14:00 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Buege OTA,Brent (2/23/2026 15:09 PST)
SIGN INFORMATION: Buege OTA,Brent (2/23/2026 15:09 PST)

IP OT Daily Documentation Entered On: 02/23/2026 14:53 PST
Performed On: 02/23/2026 14:00 PST by Buege OTA, Brent

Review/Treatments Provided
OT Therapeutic Exercise : Yes

Buege OTA, Brent - 02/23/2026 15:09 PST

Short Term Goals Reviewed : Yes
Long Term Goals Reviewed : Yes
Reassess Activities of Daily Living : Yes
Documented Pain (new) : Yes, able to self report pain
Previous Hand Dominance : Right
Current Hand Dominance : Right

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Pain Assessment
Pre Treatment Numeric Pain Rating Scale: : 8
Acceptable Level of Pain : 2
Post Treatment Numeric Pain Rating Scale: : 7
Pain Interventions Implemented : Yes

Buege OTA, Brent - 02/23/2026 14:16 PST

OT Pain Interventions
OT Pain Intervention Grid

	OT Pain Intervention #1	OT Pain Intervention #2
Activity :	Positioning/Repositioning/Elevation	Rest breaks
Activity Performed :	Yes	Yes
Response :	Minimal response to treatment	Minimal response to treatment
Comments :	As needed throughout session	As needed throughout session
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DOC - Occupational Therapy

OT Pain Intervention Grid Updated : Yes

Buege OTA, Brent - 02/23/2026 14:16 PST

IADL OT

IADL Goals Grid

Phone Use Goal : Independent

Community Mobility, Safety Goal : Independent

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IADLs Reviewed : Yes

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Therapeutic Exercise

OT Therapeutic Exercise Grid

	Exercise 1
Exercise :	Upper extremity strengthening
Exercise Performed :	Yes
Skilled Strategies :	Analysis of abilities and tolerance
Position :	Semi-Supine
Therapy Equipment :	Flexbar, Theraband
Repetition/Time :	3 x 30
Response :	Actively participated in skilled therapy
Comments :	3 LBS therabar and red flex bar
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Therapeutic Exercise Grid Updated : Yes

Buege OTA, Brent - 02/23/2026 15:09 PST

Short Term Goals

Eating Goal Grid

	Goal #1
Activity :	Eating
Assist :	IND
Status :	Goal met

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Grooming Goal Grid

	Goal #1
Activity :	Grooming routine
Assist :	IND
Status :	Goal met
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Upper Body Dressing Short Term Goal Grid

	Goal #1
Activity :	Upper extremity dressing
Assist :	IND
Status :	Goal met
Date Met :	02/19/2026 PST
	Buege OTA, Brent - 02/23/2026 14:16 PST

Lower Body Dressing Grid

	Goal #1	Goal #2
Activity :	Lower extremity dressing	Don/Doff footwear
Assist :	IND	IND
Status :	Goal met	Goal met
Date Met :	02/19/2026 PST	02/19/2026 PST
	Buege OTA, Brent - 02/23/2026 14:16 PST	Buege OTA, Brent - 02/23/2026 14:16 PST

Bathing Goal Grid

	Goal #1	Goal #2
Activity :	Bathe	Shower transfer
Assist :	IND	IND
Status :	Goal met	Progressing, continue
Date Met :	02/19/2026 PST	
Comment :	IND w bed bath	

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Toileting and Transfers Goal Grid

	Goal #1	Goal #2
Activity :	Toileting	Toilet transfers
Assist :	Sub/Max A	IND
Status :	Progressing, continue	Progressing, continue
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Long Term Goals

Self-Care Goals Grid

GG0130 Eating Goal : Independent - 06
GG0130 Oral Hygiene Goal : Independent - 06, IND on eval
GG0130 Toileting Hygiene Goal : Independent - 06
GG0170 Toilet Transfer Goal : Independent - 06
GG0130 Shower, Bathe Self Goal : Independent - 06
GG0130 Upper Body Dressing Goal : Independent - 06
GG0130 Lower Body Dressing Goal : Independent - 06
GG0130 Put On,Taking Off Footwear Goal : Independent - 06
GG0170 Car Transfer Goal : Independent - 06

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CS Self Care Goals Grid

Eating : Not met
Oral Hygiene : Met
Toileting Hygiene : Not met
Toilet Transfer : Not met
Shower, Bathe Self : Met
Upper Body Dressing : Met
Lower Body Dressing : Met
Putting On, Taking Off Footwear : Met
Car Transfer Goal : Not met

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Additional Self Care Goals Grid

Wash/Dry hands Goal : Ind
Wash/Dry face Goal : Ind
Comb/Brush hair Goal : Ind
Tub/Shower Transfer Goal : Ind

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CS Additional Self Care Goals Grid

Wash/Dry hands : Not met

DOC - Occupational Therapy

Wash/Dry face : Met
Comb/Brush hair : Not met
Tub/Shower Transfer : Not met

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Cogntive Functional Goals OT Grid
Attention Goal : Independent
Short Term Memory Goal : Independent
Safety Awareness/Insight Goal : Independent
Simple Problem Solving Goal : Independent
Complex Problem Solving Goal : Independent

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Cogntive Functional Status Goals OT Grid
Attention : Met
Short Term Memory : Met
Safety Awareness/Insight : Met
Simple Problem Solving : Met
Complex Problem Solving : Met

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OT Patient/Caregiver Goal : "Reduce pain and get healthy"

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OT IP Long Term Goals Grid

	Long Term Goal 1
Goal :	Pt will be able to teach back 2-3 non-pharmacological pain management strategies IND by d/c.
Status :	Progressing, continue
	Buege OTA, Brent - 02/23/2026 14:16 PST

IP OT LTG Documentation : Yes

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Occupational Therapy Education
Education Plan Status
Acceptable Level of Pain : Education Ongoing
Adaptive Equipment : Education Ongoing
Adaptive Techniques : Education Ongoing
ADL Training : Education Ongoing
Balance Considerations/Strategies : Education Ongoing
Bathroom/Safety Equipment : Education Ongoing

DOC - Occupational Therapy

Bed Mobility : Education Ongoing
Bed Positioning : Education Ongoing
Bladder Management : Education Ongoing
Bowel Management : Education Ongoing
Breathing Techniques : Education Ongoing
Car Transfers : Education Ongoing
Community Mobility & Safety : Education Ongoing
Discharge Plan : Education Ongoing
Education about Plan of Care : Education Ongoing
Energy Conservation : Education Ongoing
Exercise Program : Education Ongoing
Fall Prevention : Education Ongoing
Fall Recovery : Education Ongoing
Home Safety : Education Ongoing
Non-Pharmacological Pain Interventions : Education Ongoing
Pain Management : Education Ongoing
Precautions : Education Ongoing
Stairs : Education Needed
Transfer/Mobility Techniques : Education Ongoing
With Notebook : Education Ongoing
Other : Education Ongoing

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Teaching Method : Demonstration, Explanation
Additional Session Learner/s Present v2 : Patient

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Assessment

Treatment Summary/Progress : Pt semi supine in bed at time of service and agreable to treatment. pt engaged in therapuetic exercies to increase functional strength and ROM of BUE for increased safetry and IND with self care tasks. Pt declining OOB actvities despite cuing for encoruagment and education.
IP Patient Impairments/Limitations-OT : Balance deficits, Basic ADL deficits, Impaired activity tolerance, Pain limiting function, Strength deficits

Buege OTA, Brent - 02/23/2026 14:16 PST
(As Of: 02/23/2026 14:53:48 PST)

Problems(Active)

Ability to perform ADL - activity of daily living (SNOMED CT :3302763012)	*Name of Problem:* Ability to perform ADL - activity of daily living ; *Recorder:* Villano OTR/L, Shirnice; *Confirmation:* Confirmed ; *Classification:* OT ; *Code:* 3302763012 ; *Contributor System:* PowerChart ; *Last Updated:* 02/17/2026 16:58 PST ; *Life Cycle Date:* 2/17/2026 ; *Life Cycle Status:* Active ; *Vocabulary:* SNOMED CT
Activity tolerance (SNOMED CT :2156821014)	*Name of Problem:* Activity tolerance ; *Recorder:* Villano OTR/L, Shirnice; *Confirmation:* Confirmed ; *Classification:* OT ; *Code:* 2156821014 ; *Contributor System:* PowerChart ; *Last Updated:* 02/17/2026 16:58 PST ; *Life Cycle Date:* 2/17/2026

DOC - Occupational Therapy

; *Life Cycle Status:* Active ; *Vocabulary:* SNOMED CT

Plan/Therapy Intensity and Frequency Plan
OT- Frequency Hours(s)/day(s) : 1.5 hrs/day
OT- Duration : 2 weeks
OT- Frequency Day(s)/Week(s) : 5 days/week
OT Anticipated Treatments, Needs : Balance training, Basic activities of daily living, Energy conservation training, Equipment training, Focused Breathing, Group therapy, Home exercise program, Mobility training, Pain management, Pain education, Positioning/Repositioning/Elevation, Safety education, Stretching, Therapeutic exercises, Therapeutic use of self
OT Tx Plan/Goals Established w Patient : Yes
Therapy Intensity and Frequency Plan (The estimated intensity plan may fluctuate based on individual need and tolerance of the patient) : 3 Hours Per Day/5 Days Per Week
Group Therapy and Concurrent Therapy Will Better Meet the Individual Needs of the Patient in Treatment Sessions in Which the Patient Requires : Heighten complexity of dual/multi task practice for functional improvement, Practice task in a more engaging environment for functional improvement, Socialization for functional improvement

Buege OTA, Brent - 02/23/2026 14:16 PST

Treatment Session Conclusion - Patient Status
Location-Transport : Left in room

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IP OT Time Spent With Patient
IP OT Individual Charge : 60
OT Total Treatment Minutes : 60
Correct Perf On/Start Time Documented : Yes
Innovative Equipment : None
Supervising Occupational Therapist : Tanabe OT, David
IP OT Total Individual Minutes : 60

Buege OTA, Brent - 02/23/2026 14:16 PST

DOC - Physical Therapy

DOCUMENT NAME: IP PT Daily Documentation - Text
SERVICE DATE/TIME: 2/23/2026 09:00 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Thornicroft PTA,Brendan (2/23/2026 12:08 PST)
SIGN INFORMATION: Thornicroft PTA,Brendan (2/23/2026 12:08 PST)

IP PT Daily Documentation Entered On: 02/23/2026 07:27 PST
Performed On: 02/23/2026 09:00 PST by Thornicroft PTA, Brendan

Review/Treatments Provided
Reassess Mobility : Yes
Documented Pain (new) : Yes, able to self report pain
PT Therapeutic Activity,Mobility,Balance : Yes
Gait Training : Yes

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Short Term Goals Reviewed : No
Previous Hand Dominance : Right
Current Hand Dominance : Right

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Pain Assessment
Primary Pain Location : Perianal
Pre Treatment Numeric Pain Rating Scale: : 6
Acceptable Level of Pain : 6
Post Treatment Numeric Pain Rating Scale: : 6
Pain Interventions Implemented : No
Patient declined Pain Mgmt Treatment : Patient reports pain level being within acceptable level.

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Therapeutic Activities/Mobility/Balance
Therapeutic Activities Grid Updated : Yes

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PT Therapeutic Activities Grid

	Activity 1	Activity 2
Activity :	Bed mobility, Transfer training, Transitional movement	Tolerance to upright position
Activity Performed :	Yes	
Skilled Strategies :	Analysis of abilities and tolerance	Analysis of abilities and tolerance

DOC - Physical Therapy

Adaptive Equipment :	Walker - rolling	
Response :	Actively participated in skilled therapy	
Comment :	Rolling left and right and supine <> sit EOB IND, STS from EOB IND and SPT with and without FWW IND	Cuing for pressure relief while seated in w/c
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Functional Abilities

Self Care Grid

GG0170 Toilet Transfer : Independent - 06

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Mobility/Transfers Grid

GG0170 Picking Up Object : Independent - 06
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GG0170 Sit to Lying : Independent - 06
GG0170 Lying to Sitting on Side of Bed : Independent - 06
GG0170 Sit to Stand : Independent - 06
GG0170 Chair,Bed to Chair Transfer : Independent - 06

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IADL PT

IADL Goals Grid

Phone Use Goal : Independent
Community Mobility, Safety Goal : Independent

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IADLs Reviewed : Yes

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Gait Training

Gait Training Grid

	Gait Training Activity 1	Gait Training Activity 2
Activity :	Gait	Stair climbing
Activity Performed :		Yes

DOC - Physical Therapy

Skilled Strategies :	Analysis of abilities and tolerance, Designed treatment to individual needs	Analysis of abilities and tolerance, Designed treatment to individual needs
Adaptive Equipment :	Walker - rolling	
Comment :	Gait with FWW 50', SPV, cuing for energy conservation	Ascended/descended one 6" step IND, pt declined further stair training due to sensation of BM
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Gait Training Grid Updated : Yes

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Physical Therapy Education

Education Plan Status

Acceptable Level of Pain : Education Ongoing
Adaptive Equipment : Education Ongoing
Adaptive Techniques : Education Ongoing
Ambulation : Education Ongoing
Ambulatory Devices : Education Ongoing
Balance Considerations/Strategies : Education Ongoing
Bathroom/Safety Equipment : Education Ongoing
Bed Mobility : Education Ongoing
Bladder Management : Education Ongoing
Bowel Management : Education Ongoing
Breathing Techniques : Education Ongoing
Car Transfers : Education Ongoing
Community Mobility & Safety : Education Ongoing
DME Instruction & Safety : Education Ongoing
Education about Plan of Care : Education Ongoing
Energy Conservation : Education Ongoing
Exercise Program : Education Ongoing
Fall Prevention : Education Ongoing
Fall Recovery : Education Ongoing
Home Safety : Education Ongoing
Non-Pharmacological Pain Interventions : Education Ongoing
Pain Management : Education Ongoing
Precautions : Education Ongoing

DOC - Physical Therapy

Stairs : Education Needed
Transfer/Mobility Techniques : Education Ongoing
What to Expect During the Stay : Education Complete
With Notebook : Education Ongoing
Other : Education Ongoing

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Teaching Method : Demonstration, Explanation
Additional Session Learner/s Present v2 : Patient

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Assessment

Treatment Summary/Progress : Pt presents sueated in w/c and agreeable to treatment. Pt transported to gym and attempted stair training but upon completing obe step pt reported feeling urge for BM. Pt taken back to room therapist and pt stated the urge was gone but when therapist suggested gait training pt stated the urge to have a BM had returned. Therapist suggested sitting on the toilet but pt initially wanting to transfer to bed but once in bed electing to transfer to toilet. Significant time required to complete BM with pt still reporting that she felt like she hadn't voided completely. CNA present for pericare and brief change and therapist assisted with pt transfer to bed. Pt left with all needs met and CLIR. SEe grids for all details.

Impediments: progressing to continue
DME: FWW
Family training: no
Fall Prec: reviewed, appropriate--BASIC

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IP Patient Impairments/Limitations-PT : Balance deficits, Bed mobility deficits, Impaired activity tolerance, Impaired gait, Pain limiting function, Range of motion deficits, Strength deficits, Transfer deficits, Transition deficits

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Problems(Active)

Ability to perform ADL - activity of daily living (SNOMED CT :3302763012)	*Name of Problem:* Ability to perform ADL - activity of daily living ; *Recorder:* Villano OTR/L, Shirnice; *Confirmation:* Confirmed ; *Classification:* OT ; *Code:* 3302763012 ; *Contributor System:* PowerChart ; *Last Updated:* 02/17/2026 16:58 PST ; *Life Cycle Date:* 2/17/2026 ; *Life Cycle Status:* Active ; *Vocabulary:* SNOMED CT
Activity tolerance (SNOMED CT :2156821014)	*Name of Problem:* Activity tolerance ; *Recorder:* Villano OTR/L, Shirnice; *Confirmation:* Confirmed ; *Classification:* OT ; *Code:* 2156821014 ; *Contributor System:* PowerChart ; *Last Updated:* 02/17/2026 16:58 PST ; *Life Cycle Date:* 2/17/2026 ; *Life Cycle Status:* Active ; *Vocabulary:* SNOMED CT

Plan/Therapy Intensity and Frequency Plan

PT- Frequency Hours(s)/day(s) : 1.5 hrs/day
PT- Duration : 2 weeks
PT- Frequency Day(s)/Week(s) : 5 days/week

DOC - Physical Therapy

PT Anticipated Treatments, Needs : Aromatherapy, Balance training, Basic activities of daily living, Caregiver training, Gait training, Group therapy, Heat, Safety education
PT Plan/Goals Established w Pt/Caregiver : Yes
Therapy Intensity and Frequency Plan : 3 Hours Per Day/5 Days Per Week
Patient Group Therapy Need : Heighten complexity of dual/multi task practice for functional improvement, Practice task in a more engaging environment for functional improvement, Socialization for functional improvement

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Treatment Session Conclusion - Patient Status
Location-Transport : Left in room
Set-up : Bed alarm activated, Call light within reach, Bedside table within reach

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IP PT Time Spent With Patient
Correct Perf On/Start Time Documented : Yes

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PT Interventions
Functional Mobility : Yes
Gait Training : Yes
(Comment: stairs [Thornicroft PTA, Brendan - 02/23/2026 12:08 PST])

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PT Innovative Equipment : None

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IP PT Individual Charge : 60
PT Total Treatment Minutes : 60
Supervising Physical Therapist : Margolin PT, Kerry
IP PT Total Individual Minutes : 60

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